



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ربيعة عبدالله الكيلاني طة

المحافظة : الجيزة

الحي / المدينة : اوسيم

العنوان : ---

رقم البطاقة : 27312010103049 رقم التليفون : 01226723931

تاريخ تسجيل الإستمارة : 20-09-2026

العمر (تاريخ الميلاد) : 01-12-1973 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رثيفة عبدالله الكيلاني طة National ID: 27312010103049	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

MDT FORM

Name:	Raafa Abdallah Elkelany .	MRN:	475
Age:	53 Y	ID:	27312010103049
Menopausal Sta	pre ???	Phone:	01226733931
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	DM	Reason Of MDT:	post op

Clinical Data:

BX: idc2
ER: 8/8 PR: 7/8 HER2: -ve KI67: 25%

Sonomammography:

Mets Workup:

History:

prem left breast cancer multicentric ct2 n1 mo idc 2
er8/8 pr7/8 her2-ve cts hfl so st zoladex ai from 9/2025 for doubt of being
metastatic mri abdomen free hemangioma sm 3/2026 rd

PT underwent LT MRM :(post ET)
residual IDC G2
free margins
ALND 10/16)
stage ypT 1a yp N 3a

Recommendations:

MDT Outcome & Plan

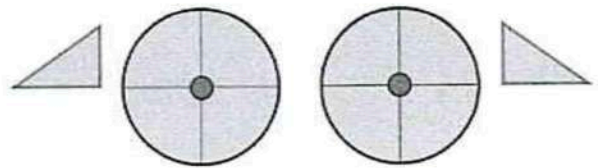
- Complete work up (CT CAP + bone scan)
- Adjuvant CTH (4 cycles of AC then 4 cycles of TXT)
- PORT to left chest wall and regional LNs.
- Femara for 7 years (ensure postmenopausal status).
- Zometa / 6 months after dental assesement
- Adjuvant CDK4/6 for 2 years From NGO .

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د محمد كبريتي					

R

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T N M

Patient Name : الأستاذة / رفيفه عبد الله الكيلاني	Request No. : 13006417
Patient No. : 27312010103049	Request Date : 10/05/2026
Path No. : 1412/26	Verification Date : 21/05/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 24x20x7 cm, with covering skin ellipse measuring 24x19 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 4x3x2 cm, located 1 cm away from the deep margin.
- * Axilla: Axillary fat measured 12x7x2 cm, entangling 16 lymph nodes, the largest measures 2x1x1 cm.
- 12 Blocks:
- 1-4: T.B.S., 5- N&A., 6-Deep margin., 7:12- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic examination:

- * Histological type: Residual viable invasive duct carcinoma.
(Actual tumor size 0.2x0.2 cm) (Cellularity 1%).
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: 2% Low grade DCIS (Solid & cribriform patterns).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive viable metastatic deposits in ten out of sixteen lymph nodes (10/16) (The largest nodal mets measuring 1.5 cm, with extra-nodal extension).
- * Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Verified by :
Dr.Duaa Mohamed



Patient Name : الأستاذ / رفيفه عبد الله الكيلاني
Patient No. : 27312010103049
Path No. : 1412/26
Sex : Female
Age : 52 Y
Referring Doc :

Request No. : 13006417
Request Date : 10/05/2026
Verification Date : 21/05/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Left breast cancer (Post-NACT), Left modified radical mastectomy:

- Residual viable invasive duct carcinoma, Grade 2 (Cellularity 1%).
- 2% low grade of ductal carcinoma in situ (Solid & cribriform patterns).

* Deep margin: Free.

* Axillary lymph nodes: Positive viable metastatic deposits in ten out of sixteen lymph nodes (10/16) (The largest nodal mets measuring 1.5 cm, with extra-nodal extension).

* Histologic response to therapy: Pathological partial response to therapy.

• Stage: ypT1a, ypN3a.

• RCB: 3.151, RCB class-II.

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed



جمهورية مصر العربية
وزارة الداخلية
مركز التوثيق - الجيزة

رابطه
عبدالله الخولاى طه على
ه ش عبدالله
مركز التوثيق - الجيزة

٢٧٣١٢٠١٠٣٠٤٩

K04134066

٢٠٢٤/٠٦ ٢٧٣١٢٠١٠٣٠٤٩
رقم التوثيق

أنش
مصلحة
مقررة
حتى محدود عبد الخولاى طه
الخطوة سارية حتى ٢٠٢٤/٠٥/٢٧

١

MDT FORM

23/ 9 / 2026

G

Name	رَيْفَة عبد الوكيل	MRN	٩٠٧٩ ١٢٥
Age	٥٢	ID	27312010103049
Menopausal status	post perimenopausal	Phone	01226 723931
NRS SCORE		Marital status	Married Nallypara
BMI		Family history	-ve
CO morbidity	DM → oral	REASON OF MDT	palliative

Clinical Data

BX:	IDC G2	R	L
ER:	8/8	PR:	7/8
HER2:	0	Ki67:	25%
Sonomammography:	multicentric malignant looking mass BIRDS 5 e path axillary LN's		
Mets Workup:	C.T Chest → free Triphasic Abd → likely mets HFL seg VII		

TNM

Recommendations: A case of oligometastatic breast cancer to liver
for - Bone scan
- Bx from HFL
- start for E2, FSH, LH

MDT OUTCOME & PLAN

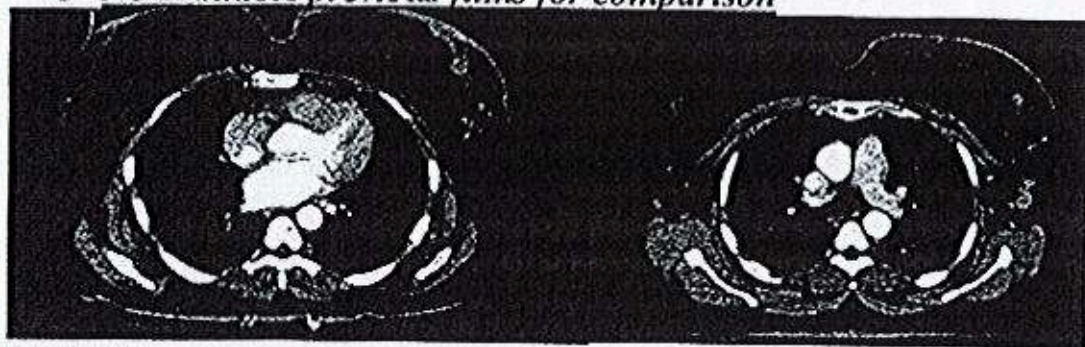
- ① Bone Scan + Biopsy h HFL.
- ② Start AI + Zoladex

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Hoss	Dr. Hoss				
23/9/2025	23/9/2025				

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

- o No available previous films for comparison



Findings:

- Left breast retroareolar speculated soft tissue mass lesion is noted measuring about 17x15 mm associated with skin thickening reaching 8.5 mm.
- Other left breast well defined lesions noted at the outer quadrant for sonomammography correlation.
- Few enlarged left axillary and subpectoral lymph nodes are noted, the largest seen measuring 20x14 mm.
- No sizable nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Mild cardiomegaly is noted.
- No evidence of pleural or pericardial effusion.

OPINION:

- Left breast retroareolar speculated soft tissue mass lesion is associated with skin thickening and other left breast outer quadrant well defined lesions for sonomammography correlation.
- Few enlarged left axillary and subpectoral lymph nodes.
- Mild cardiomegaly for echocardiography correlation.

Marina Essam, MSc
Dr.Noura Nabil,MD

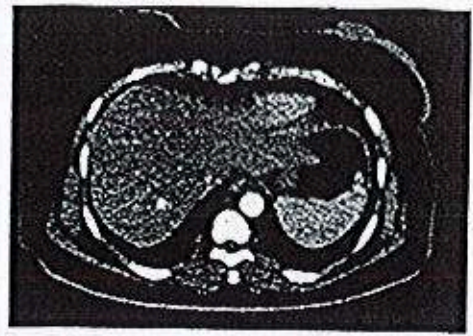
MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: -

Findings:

ver is enlarged (17 cm) with diffi low
genous CT texture showing tiny c at VII
ncing focal lesion measuring about 10x10 mm for
iphasic CT/dynamic MRI correlation.

Patent homogenously enhancing portal vein and its
branches as well as its main tributaries.

- No intra-hepatic or extra-hepatic biliary radicles
dilatation.
- Gall bladder stones are noted.
- Normal size (11 cm) and parenchymal texture of the spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure
changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no definite masses or stones.
- No ascites.
- Left adnexal cystic lesion is noted measuring 3x3 cm.



main

Opinion:

- *Fatty hepatomegaly with segment VII focal lesion for triphasic CT/dynamic MRI correlation.*
- *Gall bladder stones.*

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
11/08/2025 5:20 PM



الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
الأمراض المعدية

قسم الباثولوجي

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: رفيفه عبدالله الكيلان طه على

Age: 52years

Sex: Female

Request type: مبادره

Sub request type:

Hospital no:

Pathology No.: 2668-2025

Department: صحة المرأة

Date of Result: 14/7/2025

Referred by: د/غاده خالك

Receiving date: 7/7/2025

Nature of specimen: Tru Cut Biopsy

I.D. Number: 27312010103049

Clinical diagnosis: Left Breast multicentric Malignant Mass

Gross pathology:

Multiple grayish white tissue cores up to 0.9x0.9cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against ER- PR- Her2- Ki67

Microscopic picture:

Microscopic examiaion revealed breast core tissue showed clusters, nest and cribriform pattern of anaplastic ductal cells with moderate anaplasia and moderate tendency to form tubules dispersed among desmoplastic stroma with mild lymphocytic response. Also seen ductal carcinoma in situ component of solid and cribriform pattern of intermediate grade with focal calcification.

Immunohistochemical Result

ER: Positive Strong nuclear staining in 80% of tumor cells (Allred Score 8/8).

PR: Positive strong nuclear staining of 40% of tumor cells (Allred Score 7/8).

Her2: No membranous staining of tumor cells (Score 0) Negative.

Ki67: Positive nuclear staining of 25% of tumor cells.

Diagnosis: Left Mass, Tru Cut Biopsy:

Invasive Duct Carcinoma (NST), Grade II

Signature:

Dr.Mona Mohamed. M.D

Pathology Lab. Manager

Nermine Nabil, M.D

Patient Name English: Ralf in Abu in Alkilany Taha

2025/07/06 11:00:06 AM : Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Summary: Left breast lump.

Technique

Oblique and CC views High resolution ultrasound

Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- LIQ spiculated dense mass, measuring about 4.6x5 cm, being at 6 cm from the nipple (based on CC view).
- Associated with UOQ focal asymmetry, edematous changes and thickened skin.
- **Ultrasound** demonstrated 6:00 irregular spiculated heterogeneous hypoechoic solid mass, measuring about 3x2 cm, with 1.6 cm posterior intraductal extension.
- Another smaller yet similar mass lesions seen at 4:00 and 1:00, ranging in size from 7mm to 15 mm.
- Along with infiltrated heterogeneous parenchyma seen at 12:00 and 3:00-4:00, the largest measures about 3.7x2.7 cm.
- Associated with edematous changes and thickened skin.
- Multiple pathological axillary lymph nodes seen at level 1, 2 and 3 (infraclavicular).
- No pathological supraclavicular LNs detected.

Right breast

- Scattered fibro glandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications. UOQ benign looking nodule, likely IMLN.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Conclusion:

Left breast (BIRADS 5): Multicentric malignant looking masses, with ipsilateral pathological axillary LNs (level 1,2, and 3).

Right breast (BIRADS 2): benign findings.

Recommendations:

Core biopsy done at same sitting for histopathological correlation.

Dr. Ghada Khaled

Patient Name: Raeefa Abd Allah Al Kilany Taha Ali	Date : 01/09/2025
Exam : triphasic C.T Liver	Age : 51 Year(s)
Medical ID: 2072/2025	Pat. ID: 110889

TRIPHASIC CT SCAN OF THE ABDOMEN AND PELVIS: -

Findings:

- The liver is enlarged (17 cm) with diffuse low homogenous CT texture showing tiny segment VII enhancing focal lesion measuring about 10x8 mm showing arterail enhancement and delayed washout
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Gall bladder stones are noted.
- Normal size (11 cm) and parenchymal texture of the spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no definite masses or stones.
- No ascites.
- Left adnexal cystic lesion is noted measuring 3x3 cm.

Opinion:

- Fatty hepatomegaly with segment VII focal lesion likely metastatic for follow up.
- Gall bladder stones.

DR/Reham khedr,,MD
Signature Reham.Khader
05/09/2025 7:54 PM



MINISTRY OF HEALTH

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: رتيغه عبدالله الكيلان طه علي

Age: 52 years

Sex: Female

Request type: مياثره

Hospital no:

Department: صحة المرأة

Referred by: د/غاده خالد

Nature of specimen: Tru Cut Biopsy

Sub request type:

Pathology No.: 2668-2025

Date of Result: 14/7/2025

Receiving date: 7/7/2025

I.D. Number: 27312010103049

Clinical diagnosis: Left Breast multicentric Malignant Mass

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Multiple grayish white tissue cores up to 0.9x0.9cm long, totally submitted.

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Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against ER-PR- Her2- Ki67

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Microscopic examination revealed breast core tissue showed clusters, nest and cribriform pattern of anaplastic ductal cells with moderate anaplasia and moderate tendency to form tubules dispersed among desmoplastic stroma with mild lymphocytic response. Also seen ductal carcinoma in situ component of solid and cribriform pattern of intermediate grade with focal calcification.

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Ki67: Positive nuclear staining of 25% of tumor cells.

Diagnosis: Left Mass, Tru Cut Biopsy;

Invasive Duct Carcinoma (NST), Grade II

Signature:

Dr. Mona Mohamed. M.D

Pathology Lab. Manager

Nermine Nabil, M.D



خطاب إداري

الرقم القومي	٢٧٣١٢٠١٠٣٠٤٩
إسم المواطن	رقيقة عبدالله الكيلاني طه

رقم الطلب	٢٧٥٦٦٨٠٦
الحالة	خطاب إداري
نص الخطاب	
الإجراء المطلوب خارج البروتوكولات	Pt under adj hormonal titt
٢٢٧١٤ / ١١٣٢٣ / ٢٨٦٧٢	

د/ محمد عقاد

الإدارة العامة للمجالس الطبية المتخصصة

مدير عام

تحريراً في ٢٠٢٦-٠٩-٠٣

٢٢٧١٤ / ١١٣٢٣ / ٢٨٦٧٢

52y old ♀ Pt, Primers pausal / Ph as Lt Breast Ca.

in 2025 — CT2N, M. IDC G-II ER8/8, PR7/8

Her-ve

CTs showed HLGs (there was dubt of being metastatic) but

MEI showed that it is Hemangioma, Pt started NACT HT

(AI-2obdtx) for 7 month then underwent Lt MRM (5/2=26

PT1a N3a Mo, so MDT Decided Adj Ch (uAc then

uTat) then posth the 7 years of AI

Patient's Name:

Reefa Abdullah Al-Kilani Taha

Date:

2026-09-06

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Right Digital soft tissue mammography with high resolution sonography

revealed:

Clinical summary: Status post left MRM & ALAND, routine follow up.

Findings:

Right breast.

- Complete fatty breast density (ACR a)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.
-

Conclusion:-

- *Right breast (BIRADS 1): normal study.*
- *Clear left mastectomy bed and axilla.*

Recommendation:-

- *For annual follow up.*

Best Regards,

DR. Nehal Fouda,MD



خطاب إداري

الرقم القومي	٢٧٣١٢٠١٠٣٠٤٩
إسم المواطن	رقيقة عبدالله الكيلاني طه

رقم الطلب	٢٧٥٦٦٨٠٦
الحالة	خطاب إداري
نص الخطاب	
الإجراء المطلوب خارج البروتوكولات	Pt under adj hormonal titt
٢٢٧١٤ / ١١٣٢٣ / ٢٨٦٧٢	

د/ محمد عقاد

الإدارة العامة للمجالس الطبية المتخصصة

مدير عام

تحريراً في ٢٠٢٦-٠٩-٠٣

٢٢٧١٤ / ١١٣٢٣ / ٢٨٦٧٢

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MEI showed that it is Hemangioma, Pt started NACT HT

(AI - Zoladex) for 7 month then underwent Lt MRM (5/2/26

PT 19 N3a Mo, so MDT Decided Adj Ch (4 Ac then

4T4t) then posth the 7 years of AI

Patient's Name:

Reefa Abdullah Al-Kilani Taha

Date:

2026-09-06

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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-

Conclusion:-

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- *Clear left mastectomy bed and axilla.*

Recommendation:-

- *For annual follow up.*

Best Regards,

DR. Nehal Fouda,MD